

CONSULTATION NOTE — CARDIOLOGY FOLLOW-UP

Patient	Arjun Mehta	Date	15-Jan-2025
Age/Sex	42 Years / Male	Visit	Post-MI 4 month review
Consultant	Dr. Venkat Rao	BP	124/76 mmHg
Weight	78 kg (prev: 84 kg)	Pulse	62 bpm, regular

SUBJECTIVE

Patient reports significant improvement. No chest pain or breathlessness at rest. Can walk 3 km without symptoms. Has successfully quit smoking — last cigarette 18-Sep-2024 (day of MI). Compliant with all medications. Completed 10/12 weeks of cardiac rehabilitation.

OBJECTIVE AND INVESTIGATIONS

Test	Sep 2024	Jan 2025	Change	Status
Echo EF	35%	48%	↑ 13%	GOOD RECOVERY
LDL Cholesterol	178 mg/dL	61 mg/dL	↓ 117	At target < 70 (near target < 55)
Total Cholesterol	242 mg/dL	148 mg/dL	↓ 94	EXCELLENT
BP (mmHg)	164/98	124/76	↓ 40/22	CONTROLLED
HR (bpm)	88	62	↓ 26	CONTROLLED on Bisoprolol
HbA1c	5.8%	5.6%	Stable	NORMAL
Creatinine	1.1 mg/dL	1.0 mg/dL	Stable	NORMAL

ASSESSMENT

Excellent post-MI recovery. EF recovered from 35% to 48% — significant improvement. LDL well controlled on Atorvastatin 80mg. BP at target. Smoking cessation maintained. Dual antiplatelet therapy (DAPT) ongoing — do NOT stop until September 2025 minimum.

PLAN

- Continue all medications unchanged
- Continue DAPT (Aspirin + Clopidogrel) until at least Sep-2025 (12 months post-stent)
- LDL 61 mg/dL — good, but target for STEMI is < 55 mg/dL. Consider adding Ezetimibe 10mg if next check > 55.
- Eplerenone: EF now 48% — can discontinue after completing 3-month course (Dec 2024 — already completed)
- Repeat Echo in 6 months — if EF > 50%, reassess Bisoprolol need
- Cardiac rehab — complete remaining 2 sessions. Continue structured exercise thereafter.
- Next review in 6 months (Jul 2025) or sooner if any symptoms.